

Policy on Paediatric Services

«Hospital Name»

Document control

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Date created	_____	Date of implementation	_____
Prepared by	Paediatric In-Charge Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

A blank marked _____ must be completed before issue.

Statement of intent

Paediatric services are organised and delivered safely, by staff with age-specific competency, with active safeguards against child and neonate abduction and abuse.

1. Purpose

This policy explains how «Hospital Name» organises and delivers safe paediatric care, provides neonatal care in line with national and international guidelines, ensures staff have age-specific competency, prevents child and neonate abduction and abuse, assesses children's growth and development, educates families, and — where in scope — provides adolescent-friendly health services.

This policy does not cover general infection control, general patient assessment, or informed consent in detail — those are covered in other hospital policies.

2. Scope

This policy applies to all doctors, nursing staff, and other personnel caring for children and neonates at «Hospital Name». If «Hospital Name» does not provide paediatric services, this policy does not apply.

3. Policy standards

«Hospital Name» delivers paediatric care under written guidance that defines its scope, provides neonatal care in line with national and international guidelines, ensures doctors and nurses have age-specific competency, gives children adequate amenities, assesses

growth, development, immunisation, and nutrition, actively prevents child and neonate abduction and abuse, educates families, and — where offered — provides adolescent-friendly health services.

Staff follow the written guidance below and keep the records it requires.

4. Non-negotiable rules

- Do not provide paediatric services without written guidance covering, at minimum, patient assessment, organisation of care, and addressing special needs, and defining the scope of paediatric services offered.
- Do not deliver neonatal care that isn't in line with national and international guidelines, and do not fail to actively promote breastfeeding.
- Do not let anyone without age-specific competency — by qualification, experience, or training — care for children, whether doctor or nurse.
- Do not skip any of the four paediatric assessment components: growth, development, immunisation, and nutrition.
- Do not operate without a defined, tested rapid-response process for child or neonate abduction and abuse, adequate security and surveillance, and trained staff.
- Do not educate families about nutrition, immunisation, or safe parenting in a language they don't understand, or skip any of the mandatory nutrition sub-topics — breastfeeding, weaning, protein-energy malnutrition, and childhood obesity.

5. What we do

5.1 Organise paediatric care under written guidance

Written guidance based on standard treatment guidelines or sound clinical practice governs paediatric care. At a minimum, it covers three things: assessment of paediatric patients, organisation of care, and addressing special needs. It also defines the scope of «Hospital Name»'s paediatric services — for example, well-baby clinics, different NICU levels, or a PICU. These are examples; «Hospital Name» defines its own scope.

5.2 Deliver neonatal care to guideline standards

Written guidance for neonatal care is in line with guidelines from national and international bodies. «Hospital Name» actively promotes breastfeeding practices.

5.3 Use age-specific competency

Both doctors and nursing staff caring for children have age-specific competency, based on qualification, experience, or training, or a combination of these.

5.4 Provide adequate amenities

Adequate amenities for the care of infants and children are available — for example, a breastfeeding room or a play area. These are examples; «Hospital Name» determines what's appropriate for its paediatric population.

5.5 Carry out a complete paediatric assessment

Paediatric assessment covers four things: growth, developmental, immunisation, and nutritional assessment, done using appropriate tools — preferably validated ones — and documented. Growth charts and immunisation records are maintained and kept up to date.

5.6 Prevent child and neonate abduction and abuse

Written guidance directs how «Hospital Name» prevents child and neonate abduction and abuse. Adequate security and surveillance is in place — for example, CCTV cameras — and there's a defined rapid-response process for any eventuality. This process is tested at intervals «Hospital Name» defines, using a table-top exercise or a mock drill. Staff are trained in prevention and rapid response, and know how to recognise and escalate suspected child abuse.

5.7 Educate families

Families are educated about nutrition, immunisation, and safe parenting, in a language they understand — using educational material if helpful. Nutrition education covers breastfeeding, weaning, and aspects of malnutrition, including protein-energy malnutrition and childhood obesity. Parents are also given appropriate at-home growth-monitoring information.

5.8 Provide adolescent-friendly health care, where this service is offered

Where «Hospital Name» offers adolescent health services, these are need-based — preventive, curative, and counselling — delivered with a respectful, non-judgemental attitude throughout registration, reception, consultation, and treatment, with confidentiality maintained throughout. Services could include managing physical and mental health conditions, counselling (including premarital counselling and anti-substance-abuse counselling), preventive health check-ups, immunisation such as HPV vaccination, and health education on topics like development, nutrition, sleep, and safe sexual practices. These are examples; the specific service mix is «Hospital Name»'s own decision based on need.

6. Stop-work authority

Do not care for a child or neonate without an active, tested process to prevent abduction or abuse in place, or without staff who have age-specific competency for that patient.

Stop-work applies to routine paediatric admission and ongoing care arrangements. A child already receiving emergency care continues to receive it while the gap is escalated.

The person who stops tells the Paediatric In-Charge and the Medical Superintendent the same shift. Refusing to place a child in an unsafe care arrangement is not a disciplinary matter.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable that this policy is followed and appropriately resourced; receives stop-work notifications the same shift.
Paediatric In-Charge	Oversees paediatric services; ensures written guidance, age-specific competency, four-component assessment, abduction/abuse prevention, and family education are in place; receives stop-work notifications the same shift.
Paediatric doctors and nursing staff	Apply age-specific competency; carry out all four paediatric assessment components (growth, developmental, immunisation, nutritional); follow neonatal care guidelines; actively promote breastfeeding; educate families in a language they understand; recognise and escalate suspected child abuse.
Security/surveillance staff	Maintain CCTV and other security measures; respond to rapid-response process activations for abduction or abuse; participate in mock drills and table-top exercises at intervals defined by the organisation.

8. Quality monitoring

Monitoring area	What is monitored
Written guidance	Written guidance covering assessment, organisation of care, and addressing special needs; scope of paediatric services defined
Neonatal care guideline alignment	Written guidance for neonatal care in line with national and international guidelines; active promotion of breastfeeding practices
Age-specific competency	Doctors and nursing staff caring for children have documented age-specific competency by qualification, experience, or training
Paediatric assessment completeness	All four assessment components completed and documented for every child: growth, developmental, immunisation, and nutritional assessment

Abduction/abuse prevention	Written guidance in place; adequate security and surveillance; rapid-response process defined and tested at organisation-defined intervals; staff trained in prevention, response, and abuse recognition
Family education	Families educated in a language they understand; all four mandatory nutrition sub-topics covered (breastfeeding, weaning, protein-energy malnutrition, childhood obesity); at-home growth-monitoring information provided
Adolescent services (where in scope)	Need-based adolescent health services delivered with respectful, non-judgemental attitude and confidentiality where offered

9. Training and staff acknowledgement

All doctors, nursing staff, and other personnel caring for children and neonates shall be familiar with the written guidance applicable to their work, including neonatal care guidelines, age-specific competency requirements, the four-component paediatric assessment, abduction and abuse prevention procedures, the rapid-response process, and family education requirements.

I have read the Policy on Paediatric Services of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

10. Distribution

This policy shall be available to all doctors, nursing staff, and other personnel caring for children and neonates at «Hospital Name».

11. Abbreviations

Abbreviation	Meaning
CCTV	Closed-Circuit Television
HPV	Human Papillomavirus
NICU	Neonatal Intensive Care Unit
PICU	Paediatric Intensive Care Unit

12. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
COP.11.a	Commitment*	Section 5.1 addresses written guidance for paediatric care based on standard treatment guidelines or sound clinical practice, covering at minimum assessment, organisation of care, and addressing special needs; and defining the scope of paediatric services offered.
COP.11.b	Commitment*	Section 5.2 addresses written guidance for neonatal care in line with national and international guidelines, and the active promotion of breastfeeding practices. The phrase "actively promotes" is preserved exactly from the source — not softened.
COP.11.c	Commitment	Section 5.3 addresses age-specific competency for both doctors and nursing staff caring for children, based on qualification, experience, or training, or a combination of these. Stop-work authority (Section 6) also applies to this element.
COP.11.d	Commitment	Section 5.4 addresses adequate amenities for infants and children (e.g., breastfeeding room, play area); specific amenities are the organisation's own determination.
COP.11.e	Commitment	Section 5.5 addresses the four-component paediatric assessment: growth, developmental, immunisation, and nutritional assessment, using appropriate

		(preferably validated) tools and documented. Growth charts and immunisation records maintained and updated.
COP.11.f	Commitment*	Section 5.6 addresses written guidance for preventing child and neonate abduction and abuse, adequate security and surveillance (e.g., CCTV), a defined rapid-response process tested at intervals the organisation defines using table-top exercise or mock drill, and trained staff who can recognise and escalate suspected child abuse. No specific testing interval has been invented. Stop-work authority (Section 6) also applies to this element.
COP.11.g	Commitment	Section 5.7 addresses family education on nutrition, immunisation, and safe parenting in a language the family understands. Nutrition education covers four mandatory sub-topics: breastfeeding, weaning, protein-energy malnutrition, and childhood obesity. At-home growth-monitoring information is also provided.
COP.11.h	Excellence	Section 5.8 addresses adolescent-friendly health services where offered: need-based preventive, curative, and counselling services with a respectful, non-judgemental attitude and maintained confidentiality. Scope-conditional: applies only where the organisation offers adolescent health services.

13. Required Records/Evidence Checklist

Written guidance — paediatric services

- Written guidance for paediatric care covering assessment, organisation of care, and addressing special needs.
- Defined scope of paediatric services offered by the organisation.

Neonatal care and breastfeeding

- Written guidance for neonatal care in line with national and international guidelines.
- Evidence that breastfeeding is actively promoted.

Age-specific competency

- Evidence of age-specific competency for doctors and nursing staff caring for children (by qualification, experience, or training).

Amenities

- Evidence of adequate amenities for infants and children appropriate to the organisation's paediatric population.

Paediatric assessment records

- Assessment records documenting all four components for every child: growth, developmental, immunisation, and nutritional assessment.
- Growth charts maintained and kept up to date.
- Immunisation records maintained and kept up to date.

Abduction and abuse prevention

- Written guidance for preventing child and neonate abduction and abuse.
- Evidence of adequate security and surveillance (e.g., CCTV).
- Defined rapid-response process; records of testing at organisation-defined intervals (table-top exercise or mock drill).
- Evidence of staff training in prevention, rapid response, and recognising and escalating suspected child abuse.

Family education records

- Records of family education on nutrition, immunisation, and safe parenting, in a language the family understands.
- Evidence that all four mandatory nutrition sub-topics were covered: breastfeeding, weaning, protein-energy malnutrition, and childhood obesity.
- Evidence that at-home growth-monitoring information was provided to parents.

Adolescent services (where in scope)

- Evidence of need-based adolescent health services with a respectful, non-judgemental approach and maintained confidentiality where offered.

14. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. COP.11.
2. National and international guidelines for neonatal care (reference for COP.11.b written guidance).

Disclaimer

This policy reorganises the supplied COP.11 objective elements and approved plain-language content into policy format. The stop-work text is reproduced exactly as supplied. Mandatory requirements and their stated modal strength have been retained. The phrase "actively promotes breastfeeding" is preserved exactly — it has not been softened to "encourages" or any weaker formulation. The four paediatric assessment components (growth, developmental, immunisation, nutritional) are preserved exactly. The testing interval for the abduction/abuse rapid-response process is hospital-defined — no specific interval has been invented. The four mandatory family-education nutrition sub-topics (breastfeeding, weaning, protein-energy malnutrition, childhood obesity) are preserved. COP.11.h (adolescent services, Excellence level) is presented as scope-conditional.